

Multidomain burden among midlife and older women across seven international ageing cohorts: a harmonized descriptive audit and cautionary profile-stability analysis

Feifan Lu^{1†}, Jun Chen^{2†}, Jizi Shen^{1†}, Li Li^{1*}, Rui Guan^{1*}

^{1*}Department of Obstetrics and Gynecology, Changhai Hospital, Naval Medical University, Shanghai, 200433, Shanghai, China.

²Department of General Surgery, The 989 Hospital of the PLA Joint Service Support Force, No.2 Huaxia West Road, Luoyang, 471031, China.

*Corresponding author(s). E-mail(s): 13764260296@163.com; cngreen785@163.com;

Contributing authors: luff94@163.com; chenjunsmmu@163.com; 664331614@qq.com;

[†]Feifan Lu, Jun Chen and Jizi Shen contributed equally to this work.

Abstract

Background: Women’s health research in later life increasingly needs multidomain measures that capture physical functioning, cognition, affective symptoms and chronic disease burden. Categorical profiles may help communicate heterogeneous burden patterns, but they can be overinterpreted when harmonization, complete-case selection, endpoint coupling and model stability are not explicit. **Methods:** We analysed approved, downloaded, de-identified cohort datasets for women aged 50 years or older in CHARLS, ELSA, HRS, KLoSA, LASI, MHAS and SHARE. The authors cleaned the local cohort files, used harmonized ageing-study variables where available [1] and locally assembled domain inputs when required. Four within-cohort burden domains were oriented so that higher scores indicated worse burden. We treated Gaussian mixture models as descriptive partitions and audited measurement comparability, denominator attrition, LFO functional-change associations, algorithm concordance, formal secondary mortality models, all-sex interaction screens and prediction guardrails.

Results: The source screen included 79,938 women, of whom 76,293 had complete four-domain descriptive assignments. Strict-core construction comprised CHARLS, ELSA, HRS and MHAS (29,058 women); SHARE, KLoSA and LASI remained sensitivity or descriptive tiers. All selected full-covariance four-domain Gaussian mixture solutions triggered near-singular covariance diagnostics, and cross-method agreement was limited in several cohorts. In strict-core LFO analyses, functional-change risk gradients were observed within cohort, but continuous three-domain scores matched or exceeded categorical LFO profiles for discrimination, model fit and decision-curve net benefit in 15 of 20 strict-core threshold comparisons. Secondary mortality Cox models showed high-burden class gradients, but proportional-hazards/time-drift flags in several cohorts and incomplete hard-endpoint harmonization kept mortality secondary. **Conclusions:** For midlife and older women’s health research, the defensible contribution is a harmonized descriptive audit and cautionary profile-stability analysis using applied-for, downloaded cohort data cleaned by the authors. Categorical profiles may support burden mapping and communication, but the current evidence does not support stable latent endotypes, transportable categorical risk tools, population prevalence claims or prediction superiority over continuous domain scores. Continuous domain scores should remain the preferred analytic scale for functional-change modelling in this package.

Keywords: aging, women’s health, multimorbidity, functional change, frailty, intrinsic capacity, harmonization

1 Background

Women’s health research has historically emphasized reproductive health, cancer, mental health and selected chronic conditions, but health after midlife also depends on how functional ability, cognitive reserve, affective symptoms and chronic disease burden accumulate together. Functional ability, intrinsic capacity and multimorbidity are central to health-system planning for ageing populations [2–5]. Frailty phenotypes and deficit-accumulation indices summarize vulnerability [6–8], but they can compress clinically different domain patterns into a single severity continuum.

This study is intentionally scoped to women aged 50 years or older. This midlife-and-older threshold captures the transition into later-life health-risk accumulation while retaining comparability across the available ageing cohorts. Women have longer survival, higher late-life disability burden and different patterns of affective symptoms,

13 multimorbidity and care needs [9]. Exploratory all-sex LFO severity-by-sex interaction
14 screens were fitted on a separately standardized all-sex scale; only one of six modelled
15 cohorts had a nominal interaction signal, so these analyses do not support a con-
16 sistent women-specific mechanism. The women-only design is therefore framed as a
17 focused women’s-health population study, not as evidence that the observed patterns
18 are unique to women.

19 Recent multidomain trajectory work suggests that functional, cognitive, mood and
20 chronic-disease indicators can evolve jointly [10]. The unresolved clinical and epi-
21 demologic problem is whether categorical burden profiles remain interpretable after
22 harmonization risk, complete-case selection, endpoint coupling and model instability
23 are made explicit. We therefore focus on conservative description and claim calibration:
24 whether multidomain burden can be summarized without overstating transportable
25 latent subtypes or prediction performance.

26 **2 Methods**

27 **2.1 Cohorts and analytic population**

28 After registration or data-use application as required by each study, we downloaded
29 de-identified cohort datasets from CHARLS, ELSA, HRS, KLoSA, LASI, MHAS and
30 SHARE official data access systems [11–17]. The authors cleaned the local cohort files,
31 retained project-specific derived outputs and assembled the present analysis dataset.
32 Variables included harmonized ageing-study products where available [1] and locally
33 assembled domain inputs when no directly comparable cleaned variable was avail-
34 able. The analytic population was women aged 50 years or older at the cohort-specific
35 selected baseline or analysis wave; we describe this population as midlife and older
36 women rather than as a uniformly geriatric population. Sex coding was confirmed
37 from local Working Data Stata value labels and do-files as **ragender=0** for women
38 and **ragender=1** for men. Source-screen, complete-domain, descriptive-profile and

functional-association denominators were separated in line with observational-study reporting principles [18]. Detailed cohort construction, harmonization and baseline covariate metadata are provided in Additional file 1.

2.2 Domain construction and harmonization review

Four domains were constructed: functional limitation, cognitive burden, affective symptoms and cardiometabolic/chronic disease burden. Functional items were anchored to activities of daily living and instrumental activities of daily living constructs where available [19, 20]. Affective symptoms used CES-D-family information where available [21]; SHARE used EURO-D information [22]. Scores were standardized within cohort and oriented so that higher values indicated worse burden. Retrospective harmonization review treated similar orientation as insufficient evidence for item identity or transportable measurement equivalence [23, 24].

2.3 Evidence tiers and descriptive profiles

The strict-core descriptive construction tier comprised CHARLS, ELSA, HRS and MHAS. SHARE was retained as construction-descriptive but validation-downgraded sensitivity evidence, KLoSA as functional-bridge sensitivity and LASI as baseline-only descriptive evidence. Cohort-specific Gaussian mixture models with two to five classes were fit to the four domain scores as descriptive clustering tools [25]. Models used full covariance matrices, `reg_covar=1e-6`, `max_iter=500`, `n_init=5` and random state 20260601. The selected model used the lowest BIC among converged models with minimum class size at least 5%, with class enumeration interpreted cautiously [26, 27]. The final stability guardrail used 200 nonparametric bootstrap refits per selected solution, covariance diagnostics and cross-algorithm agreement; these were treated as main guardrails rather than supplemental technicalities. Detailed profile dictionaries and stability outputs are provided in Additional file 2.

64 **2.4 Functional-change association and unresolved endpoint** 65 **guardrails**

66 The functional-change endpoint was deterioration of at least 0.5 SD from baseline.
67 Because functional change is a within-domain endpoint and the original four-domain
68 descriptive profiles include baseline function, this analysis cannot be interpreted as
69 independent hard-outcome validation. We therefore rebuilt exploratory LFO profiles
70 after omitting the baseline functional domain and compared categorical LFO pro-
71 files with continuous cognitive, affective and cardiometabolic/chronic scores. Models
72 adjusted for age, education, marital status, smoking and drinking. LFO model outputs
73 are provided in Additional file 3. We additionally fitted formal secondary all-cause
74 mortality Cox models, post-baseline hospitalization candidate models where cleaned
75 binary headers existed, and calibration/decision-curve guardrails; these secondary end-
76 point and prediction guardrail outputs are provided in Additional file 4. Exploratory
77 all-sex LFO severity-by-sex interaction models and survey-design audits are provided
78 in Additional file 5. These analyses were treated as secondary or sensitivity evidence
79 rather than harmonized primary validation.

80 **3 Results**

81 **3.1 Denominator lock, baseline profile and complete-case** 82 **selection**

83 The source screen included 79,938 women aged 50 years or older, and 76,293 had com-
84 plete four-domain descriptive assignments. The strict-core construction set comprised
85 29,058 women from CHARLS, ELSA, HRS and MHAS. The sensitivity/descriptive
86 construction set comprised 47,235 women from SHARE, KLoSA and LASI. These
87 counts and percentages are analytic-sample summaries rather than survey-weighted
88 population estimates. Complete-case selection was not neutral: excluded participants

Table 1 Baseline clinical profile and analytic denominators

Cohort	Tier/wave	Source/ complete N	LFO N/events/ follow-up	Baseline clinical profile	Excluded age delta
CHARLS	Strict- core; w1	6,878 / 6,019 (87.5%)	5,691; 1,766 (31.0%); 9.0 y	Age 61.9; BMI 23.9; chronic 0.7; 2+ chronic 17.6%; smoke/drink 6.5/15.0%	+4.2 y
ELSA	Strict- core; w1	6,292 / 6,104 (97.0%)	5,153; 1,316 (25.5%); 12.0 y	Age 65.1; BMI unavailable; chronic 0.7; 2+ chronic 14.9%; smoke/drink 18.2/85.2%	+6.6 y
HRS	Strict- core; w5	11,005 / 10,202 (92.7%)	9,431; 3,546 (37.6%); 15.0 y	Age 67.2; BMI 27.0; chronic 1.0; 2+ chronic 25.2%; smoke/drink 14.4/39.8%	+10.4 y
KLoSA	Bridge sensitivity; w3	4,344 / 4,081 (93.9%)	3,834; 1,144 (29.8%); 10.0 y	Age 66.0; BMI 23.4; chronic 0.7; 2+ chronic 17.4%; smoke/drink 3.0/17.7%	+4.0 y
LASI	Baseline- only; no wave field	28,165 / 27,433 (97.4%)	Unavailable in current cleaned pass	Age 62.6; BMI 23.1; chronic 0.6; 2+ chronic 13.5%; smoke/drink 3.7/2.6%	+6.5 y
MHAS	Strict- core; w1	7,440 / 6,733 (90.5%)	5,443; 1,437 (26.4%); 16.0 y	Age 62.4; BMI 27.7; chronic 0.7; 2+ chronic 13.0%; smoke/drink 9.5/17.6%	+4.5 y
SHARE	Validation down; w1	15,814 / 15,721 (99.4%)	12,205; 1,525 (12.5%); 11.0 y	Age 64.8; BMI 26.2; chronic 0.8; 2+ chronic 22.1%; smoke/drink 15.1/59.0%	+3.3 y

Source denominators are women aged 50 years or older in the selected baseline screen. Complete-domain denominators are four-domain descriptive assignments. Endpoint availability is counted before the covariate-complete LFO modelling restrictions used in the LFO association analysis and Figure 1. Education, marital status, smoking, drinking and rural/region field availability is reported in Additional file 1.

89 were older than included complete-domain participants in every cohort, including
90 HRS (+10.4 years) and ELSA (+6.6 years). Table 1 and Figure 1 separate source,
91 complete-domain, endpoint-availability and LFO model denominators. Each down-
92 stream analysis used its own endpoint- and covariate-complete denominator, so LFO,
93 mortality and descriptive-construction denominators should not be interchanged.

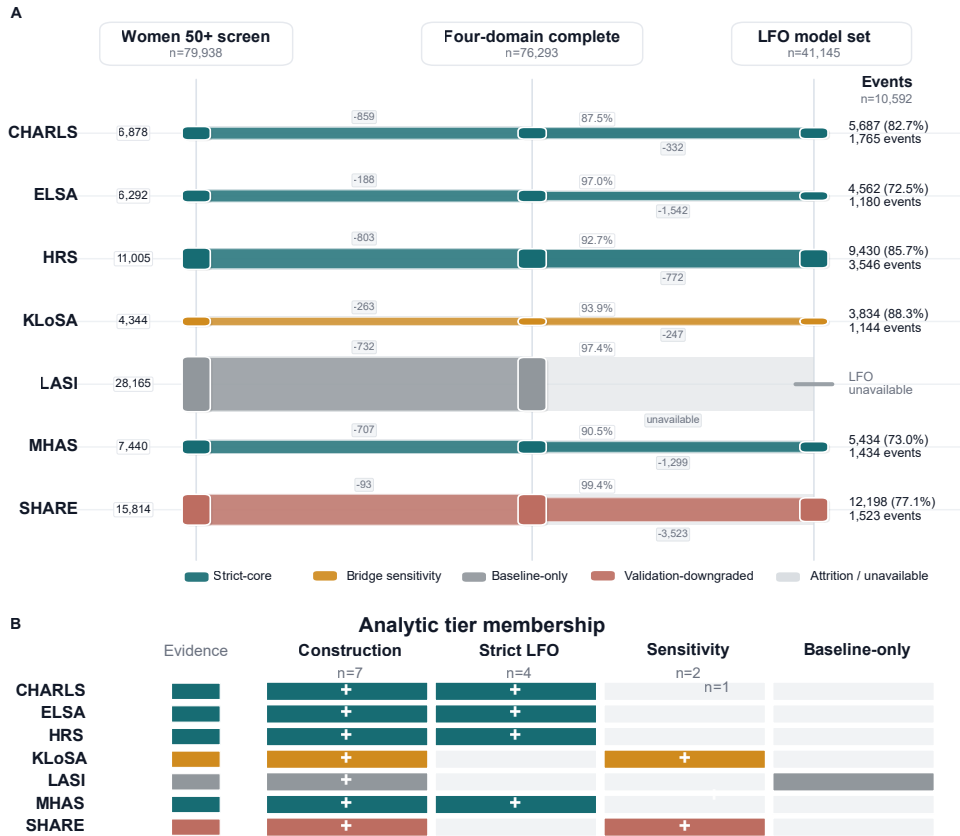


Fig. 1 Analytic denominator flow and evidence-tier membership. Panel A shows alluvial-style per-cohort attrition from women aged 50 years or older source screening to complete four-domain construction and covariate-complete LFO model denominators. These model-complete LFO denominators are intentionally smaller than the endpoint-available denominators in Table 1. Panel B summarizes analytic-tier membership for construction, strict-core LFO association, sensitivity and baseline-only descriptive roles. LASI remains baseline-only in the current cleaned-data pass.

3.2 Profile-stability guardrails

All selected full-covariance four-domain Gaussian mixture solutions triggered near-singular covariance diagnostics, with minimum covariance eigenvalues pinned at the regularization floor in at least one component per cohort. In the 200-refit bootstrap guardrail, all refits converged, but lower-tail agreement remained weak in several cohorts: p10 ARI was 0.35 in ELSA, 0.28 in HRS, 0.23 in KLoSA and 0.37 in SHARE. Even cohorts with moderate-to-high median ARI were downgraded by near-singular

101 covariance and weak lower-tail bootstrap agreement. The profile labels are therefore
 102 descriptive partitions for summarizing observed burden combinations, not stable latent
 103 disease entities or transportable risk tools (Table 2; Figure 2). The strict-core profile-
 104 pattern heatmap is retained as a supplementary descriptive display rather than as
 105 primary validation evidence.

Table 2 Model-stability guardrails for selected four-domain profile solutions

Cohort	Evidence tier	k	Bootstrap ARI	Cross-method ARI	Max condition	Claim status
CHARLS	Strict-core	3	1.00 (1.00; 0.43)	0.26 (0.12-1.00)	1.20e+06	Descriptive only
ELSA	Strict-core	5	0.84 (0.35; 0.23)	0.25 (0.22-1.00)	1.35e+06	Descriptive only
HRS	Strict-core	5	0.76 (0.28; 0.27)	0.20 (0.17-0.92)	1.52e+06	Descriptive only
KLoSA	Bridge sensitivity	3	0.98 (0.23; 0.21)	0.62 (0.22-0.93)	1.36e+06	Bridge sensitivity
LASI	Baseline-only	3	0.99 (0.97; 0.16)	0.22 (0.09-0.96)	1.19e+06	No follow-up validation
MHAS	Strict-core	5	0.98 (0.84; 0.33)	0.17 (0.15-0.95)	2.53e+06	Descriptive only
SHARE	Validation-downgraded	4	0.37 (0.37; 0.36)	0.22 (0.19-0.99)	9.28e+05	Downgraded

ARI = adjusted Rand index. Bootstrap ARI is median (p10; minimum) across 200 non-parametric refits. Cross-method ARI is median (range) against diagonal/tied GMM, k-means, sampled hierarchical clustering and continuous-severity tertiles. All selected full-covariance solutions triggered near-singular covariance diagnostics; therefore these profiles are descriptive partitions and not stable latent subtypes.



Fig. 2 Model-stability guardrails for selected descriptive profile partitions. Panel A summarizes 200-refit bootstrap ARI distributions, with higher values indicating greater refit stability. Panel B shows cross-method ARI against the selected full-covariance GMM across alternative clustering and continuous-severity comparators. Panel C ranks cohorts by log10 covariance condition number with the near-singular threshold shown as a reference line. Panel D maps bootstrap p10 ARI against log10 covariance condition to separate reproducible, fragile and near-singular descriptive solutions; it is a visualization of numerical guardrails, not a separate statistical model.

3.3 Exploratory LFO functional-change associations

Strict-core LFO functional-change analyses included 25,113 women and 7,925 functional deterioration events. Highest crude-risk LFO classes showed within-cohort functional-change gradients, although the ELSA highest-class contrast was not statistically conclusive. Continuous three-domain scores matched or outperformed categorical LFO profiles for discrimination and model fit in every strict-core cohort (Table 3;

Figure 3). Random-effects heterogeneity for the strict-core adjusted LFO risk ratios was considerable (pooled RR 1.40, 95% CI 1.19 to 1.64; I^2 80.2%; Q $p=0.002$; Additional file 3). With heterogeneity near 80%, the pooled estimate is a descriptive cross-cohort summary across non-identical instruments rather than a transportable effect estimate. These results describe within-cohort association with a functional change-score endpoint; they do not validate the four-domain descriptive profile patterns and do not demonstrate prediction superiority. SHARE and KLoSA are provided only as sensitivity rows because SHARE remained validation-downgraded and KLoSA used a functional bridge.

Table 3 Exploratory leave-functional-domain-out functional-change associations in strict-core cohorts

Cohort	LFO N/events	Risk contrast	Adjusted RR	Delta AUC	Reading
CHARLS	5,687/1,765 (31.0%)	C1 21.5% to C3 39.8%; RD 18.3 pp	1.63 (1.35 to 1.96)	-0.015	Continuous favoured
ELSA	4,562/1,180 (25.9%)	C1 21.1% to C5 42.2%; RD 21.1 pp	1.15 (0.96 to 1.37)	-0.007	Continuous favoured
HRS	9,430/3,546 (37.6%)	C1 28.4% to C4 51.8%; RD 23.4 pp	1.28 (1.16 to 1.41)	-0.008	Continuous favoured
MHAS	5,434/1,434 (26.4%)	C1 20.6% to C3 36.6%; RD 16.0 pp	1.61 (1.42 to 1.82)	-0.002	Continuous favoured

LFO = leave-functional-domain-out; RD = absolute risk difference between class 1 and the highest crude-risk LFO class. LFO classes were rebuilt without the baseline functional domain and are not the same objects as the four-domain descriptive profile patterns. Adjusted risk ratios use modified Poisson models with robust standard errors and adjustment for age, education, marital status, smoking and drinking. Delta AUC is profile AUC minus continuous-score AUC; negative values favour continuous three-domain scores. Full profile and continuous AUC values, SHARE/KLoSA sensitivity rows and class-level risks are provided in Additional file 3.

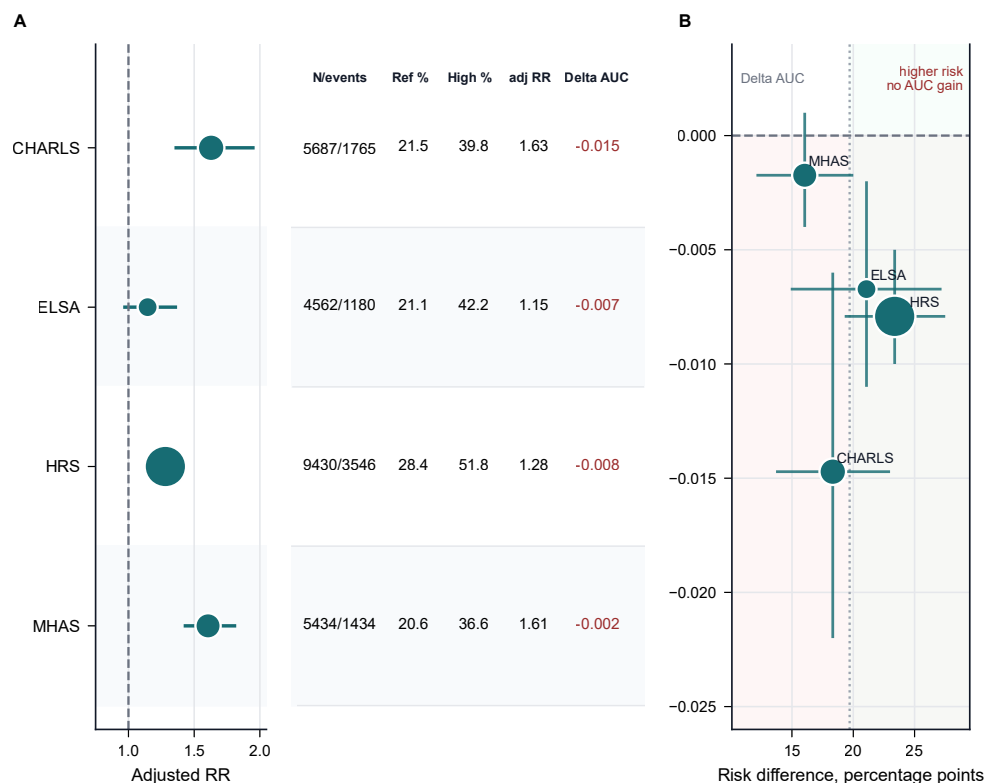


Fig. 3 Exploratory strict-core LFO functional-change associations. Panel A shows adjusted risk ratios comparing the highest crude-risk LFO class with the reference class, with point size scaled by model denominator and the adjacent table reporting N/events, reference-class risk, highest-class risk, adjusted RR and delta AUC. Panel B places cohorts in risk-difference versus delta-AUC space; horizontal intervals show uncertainty in crude risk difference and vertical intervals show 95% bootstrap intervals for delta AUC. Delta AUC is profile AUC minus continuous-score AUC, so negative values favour continuous three-domain scores.

3.4 Secondary mortality and prediction-guardrail analyses

Formal secondary all-cause mortality Cox models were fitted in six cohorts; LASI remained unavailable for mortality follow-up in the current cleaned pass. Highest-burden profile classes were associated with higher all-cause mortality in the modelled cohorts, but proportional-hazards or time-drift guardrail flags appeared in ELSA, HRS, KLoSA and SHARE. The ELSA death count was low relative to the nominal follow-up interval and may reflect incomplete mortality ascertainment in the current cleaned pass, so the ELSA hazard ratio should be read with particular caution.

129 Among strict-core cohorts, random-effects heterogeneity for the highest-class mortal-
 130 ity HR was considerable (pooled HR 2.37, 95% CI 1.99 to 2.82; I^2 78.7%; Q $p=0.003$;
 131 Additional file 4). With considerable heterogeneity and PH/time-drift flags in sev-
 132 eral cohorts, the pooled HR and flagged cohort-specific HRs are descriptive guardrail
 133 summaries rather than constant-hazard or transportable effect estimates. Mortality
 134 is therefore reported as a secondary non-circular endpoint guardrail rather than as
 135 primary validation evidence (Table 4). Decision-curve guardrails also supported the
 136 main cautionary interpretation: continuous three-domain scores had higher apparent
 137 in-sample net benefit than categorical LFO profiles in 15 of 20 strict-core threshold
 138 comparisons, without optimism correction. Calibration outputs are likewise reported
 139 only as apparent in-sample diagnostics and are not interpreted as validated calibration.

140 **3.5 Harmonization, hard-outcome and survey-design guardrails**

141 The harmonization matrix remained central to interpretation (Figure 4). Functional-
 142 domain strictness differed across cohorts: CHARLS used IADL-only information,
 143 HRS used ADL-only information, KLoSA used a bridge proxy and SHARE remained
 144 validation-downgraded. Cognitive batteries were cohort-specific, SHARE affective
 145 symptoms used EURO-D, and cardiometabolic/chronic disease indicators differed
 146 in lipid/cholesterol availability. Raw/codebook/dofile auditing identified candidate
 147 weight, primary sampling unit and strata triplets in HRS, KLoSA, LASI and SHARE,
 148 but no codebook-confirmed harmonized seven-cohort survey-design triplet was avail-
 149 able, so no survey-weighted prevalence claim is made. The all-sex interaction review
 150 found one nominal LFO severity-by-sex interaction among six modelled cohorts, which
 151 supports the focused women-only scope but not a consistent women-specific mech-
 152 anism. Hospitalization candidate models were feasible only where cleaned binary
 153 headers existed and remain header-only sensitivity evidence until codebook mapping is
 154 completed; institutionalization and care-dependence endpoints remained unavailable.

Table 4 Secondary all-cause mortality Cox guardrail models

Cohort	Tier	N/deaths; time	Highest class HR	PH/time flag	Reading
CHARLS	strict-core	5,868; 703 deaths (12.0%); 9.0 y	C3 HR 1.82 (1.54 to 2.16)	No flag	Secondary
ELSA	strict-core	4,639; 317 deaths (6.8%); 12.0 y	C5 HR 3.07 (2.15 to 4.39)	Flagged	Time-averaged
HRS	strict-core	10,043; 5,569 deaths (55.5%); 15.9 y	C5 HR 2.60 (2.34 to 2.89)	Flagged	Time-averaged
KLoSA	bridge sens.	3,990; 726 deaths (18.2%); 10.0 y	C3 HR 1.30 (1.09 to 1.56)	Flagged	Bridge sensitivity
LASI	baseline-only	unavailable	NA	NA	Not modelled
MHAS	strict-core	6,477; 2,231 deaths (34.4%); 17.0 y	C3 HR 2.39 (2.10 to 2.72)	No flag	Secondary
SHARE	validation-down	12,952; 3,198 deaths (24.7%); 11.0 y	C4 HR 2.70 (2.40 to 3.03)	Flagged	Time-averaged

These are formal secondary all-cause mortality Cox models using available death timing fields after local data cleaning. They are reported as non-circular endpoint guardrails, not as primary validation, because proportional-hazards or time-drift flags were present in several cohorts, survey-design variables were not applied, and LASI mortality follow-up was unavailable in the current cleaned pass. The ELSA death count appeared low relative to nominal follow-up and may reflect incomplete mortality ascertainment in the current cleaned pass; the ELSA HR should therefore be interpreted with particular caution. HRs in flagged cohorts should be read as conventional time-averaged guardrail summaries rather than constant-hazard effects. Technical flag coding and AIC guardrail details are provided in Additional file 4.

155 Table 5 summarizes the clinical interpretation and non-claims for a women's health
156 readership. The full workflow and supplementary profile displays are embedded after
157 the references as Supplementary Figures S1-S5.

CHARLS	Partial 99.1% IADL only	Cohort-specific 87.6% cognitive score	Primary 92.8% CES-D family	Primary 99.3% chronic count
ELSA	Primary 98.5% ADL/IADL	Cohort-specific 97.5% cognitive score	Primary 97.1% CES-D family	Primary 99.9% chronic count
HRS	Partial 99.8% ADL only	Cohort-specific 92.8% cognitive score	Primary 92.8% CES-D family	Primary 100.0% chronic count
KLoSA	Bridge 100.0% grip/falls proxy	Cohort-specific 94.7% cognitive score	Primary 99.1% CES-D family	Primary 100.0% chronic count
LASI	Primary 99.7% ADL/IADL	Cohort-specific 98.8% cognitive score	Primary 97.4% CES-D family	Primary 99.8% chronic count
MHAS	Primary 93.9% ADL/IADL	Cohort-specific 92.7% cognitive score	Primary 93.7% CES-D family	Primary 97.9% chronic count
SHARE	Primary 100.0% ADL/IADL	Cohort-specific 99.5% cognitive score	Alternate instrument 100.0% EURO-D	Primary 99.5% chronic count
	Functional	Cognitive	Affective	Cardiometabolic/ chronic
	Primary source	Cohort-specific / partial cognition	Partial or alternate instrument	Bridge proxy

Fig. 4 Cohort-domain harmonization risk matrix. Cells summarize source tier, non-missing percentage and the principal construct used for each burden domain. The matrix is measurement-risk evidence rather than a biological result.

Table 5 Clinical interpretation and non-claims for women’s health research

Finding	Interpretation for midlife and older women’s health	What should not be concluded
Four-domain burden patterns can be described across seven cohorts	Functional limitation, cognitive burden, affective symptoms and cardiometabolic/chronic burden can be mapped jointly rather than treated as isolated conditions.	The profiles are not instrument-identical, survey-weighted population prevalence estimates or proof of universal cross-national classes.
Categorical profiles were numerically fragile	Profile labels are useful as descriptive communication aids and for auditing heterogeneity.	The selected Gaussian mixture classes should not be interpreted as stable latent endotypes or transportable clinical risk tools.
Continuous LFO domain scores matched or exceeded categorical profiles for functional-change modelling	Continuous cognitive, affective and cardiometabolic/chronic burden scores should remain the preferred analytic scale for functional-change models in this package.	The analysis does not show categorical prediction superiority or justify replacing continuous multidomain scores with classes.
Functional-change gradients and secondary mortality gradients were observed	The direction of association is clinically coherent and supports further work with independent follow-up endpoints.	Functional change remains partly coupled to baseline functional constructs, and mortality is a secondary guardrail with proportional-hazards/time-drift flags.
Women-only analyses were prespecified for this descriptive package	The study provides a focused midlife-and-older women’s health map of multidomain burden and its measurement caveats.	The all-sex interaction screen does not establish a consistent women-specific biological mechanism.

LFO = leave-functional-domain-out. This table is a claim-calibration summary derived from the main analyses and supplementary guardrail audits; it is not a separate statistical model.

158 4 Discussion

159 This manuscript is deliberately scoped as a harmonized descriptive audit and cau-
160 tionary profile-stability analysis for midlife and older women’s health research. The
161 seven-cohort comparison shows that multidomain burden among women aged 50 years
162 or older can be summarized and audited, but the selected categorical partitions are
163 not robust enough to be interpreted as stable endotypes or transportable risk tools.
164 The main positive contribution is transparent measurement and denominator mapping
165 across several major ageing cohorts; the main cautionary finding is that categorical
166 profile solutions are numerically fragile and do not improve on continuous domain
167 scores for the current functional-change endpoint.

168 The functional-change analysis should be interpreted conservatively. The LFO
169 approach reduces predictor-side endpoint coupling by removing baseline function
170 from the profile-construction step, but the outcome remains a within-domain change
171 score rather than an independent hard clinical endpoint. The observed risk gradi-
172 ents therefore support exploratory within-cohort association, not validation of the
173 four-domain descriptive patterns. This limitation is especially important because the
174 formal secondary mortality analyses show that clinically more independent endpoints
175 are partly feasible, while also demonstrating why mortality must remain secondary
176 in this package because several cohorts triggered proportional-hazards or time-drift
177 guardrails.

178 The women-only scope is also conservative. The analysis addresses midlife and older
179 women as a clinically important population with high late-life disability burden, but
180 it does not establish sex-specific mechanisms. Exploratory all-sex LFO severity-by-sex
181 interaction models were fitted on a separately standardized all-sex scale and showed
182 no consistent cross-cohort interaction signal; only CHARLS had a nominal interac-
183 tion. These models were not used for women-only profile construction or outcome
184 modelling. A stronger mechanistic women’s-health paper would require a prespecified

all-sex design, formal sex-interaction estimands and harmonized women-specific exposures. The present implication is narrower: women’s health studies after midlife should report multidomain burden with transparent denominator, harmonization and stability guardrails, and should retain continuous domain scores when modelling functional change.

5 Strengths and limitations

Strengths include the seven-cohort scope, codebook-confirmed sex coding, explicit denominator locking, separation of strict-core primary evidence from sensitivity tiers, baseline clinical characterization, included-versus-excluded missingness auditing, item-level harmonization review, LFO functional-change modelling, formal secondary mortality modelling, all-sex interaction screens and 200-refit algorithm-stability diagnostics. Limitations remain substantial. Domain measures were harmonized by orientation and within-cohort standardization but were not instrument-identical across cohorts; this is particularly important for Table 3 because functional instruments differed by cohort. Complete-case selection likely removed older and higher-burden participants. Survey weights, strata and primary sampling units were screened using cleaned headers, Stata metadata and dofile/codebook text, but no common codebook-confirmed seven-cohort design triplet was available; the analyses are therefore not survey-weighted population estimates. Functional change remained a coupled within-domain endpoint. LASI lacked a follow-up validation denominator, SHARE was validation-downgraded and KLoSA used a functional bridge. Mortality was modelled only as secondary evidence; hospitalization models remain header-only candidate analyses; institutionalization and care-dependence endpoints remain unavailable. The current study therefore cannot claim population prevalence, stable latent endotypes, sex-specific mechanisms, independent hard-outcome validation or superiority over continuous domain scores.

211 **6 Conclusions**

212 Across seven international ageing cohorts, multidomain burden among midlife and
213 older women can be described, tiered and audited. The conclusion is cautionary:
214 categorical profiles are descriptive partitions with important measurement, stabil-
215 ity and endpoint-coupling caveats. For functional-change modelling in this package,
216 continuous domain scores remain the preferred analytic scale.

217 **Abbreviations**

218 ADL, activities of daily living; AIC, Akaike information criterion; ARI, adjusted Rand
219 index; BIC, Bayesian information criterion; CES-D, Center for Epidemiologic Studies
220 Depression Scale; CHARLS, China Health and Retirement Longitudinal Study; ELSA,
221 English Longitudinal Study of Ageing; GMM, Gaussian mixture model; HRS, Health
222 and Retirement Study; IADL, instrumental activities of daily living; KLoSA, Korean
223 Longitudinal Study of Aging; LASI, Longitudinal Aging Study in India; LFO, leave-
224 functional-domain-out; MHAS, Mexican Health and Aging Study; SHARE, Survey of
225 Health, Ageing and Retirement in Europe.

226 **Declarations**

227 **Ethics approval and consent to participate**

228 This study used approved, downloaded, de-identified cohort datasets from CHARLS,
229 ELSA, HRS, KLoSA, LASI, MHAS and SHARE after registration or data-use appli-
230 cation as required by each cohort. Each source cohort obtained ethics approval and
231 participant consent according to its protocol. The authors cleaned and harmonized the
232 analysis files locally. The present analysis did not involve direct participant contact.

233 **Consent for publication**

234 Not applicable.

235 **Availability of data and materials**

236 The source cohort datasets are available through their respective official data portals
237 or data access systems subject to registration, application approval and data-use con-
238 ditions. The authors downloaded, cleaned and harmonized the approved de-identified
239 cohort files for this analysis. Source and derived participant-level data cannot be redis-
240 tributed if doing so would violate source cohort data-use agreements. Analysis code,
241 harmonization metadata and aggregate non-identifying outputs used to reproduce the
242 manuscript tables and figures are archived at Zenodo: DOI [10.5281/zenodo.20589522](https://doi.org/10.5281/zenodo.20589522),
243 with the development version available at GitHub: [https://github.com/Luciky-Leo/](https://github.com/Luciky-Leo/older-women-multidomain-burden)
244 [older-women-multidomain-burden](https://github.com/Luciky-Leo/older-women-multidomain-burden).

245 **Competing interests**

246 The authors declare that they have no competing interests.

247 **Funding**

248 No specific funding was declared for this study.

249 **Authors' contributions**

250 FL, JC and JS contributed equally to this work and share first authorship. FL, JC,
251 JS, LL and RG contributed to study conception and design, interpretation of findings,
252 manuscript drafting and critical revision. FL and RG led data harmonization and
253 statistical analysis. LL and RG supervised the work; RG is the primary corresponding
254 author and LL is a co-corresponding author. All authors read and approved the final
255 manuscript.

256 Acknowledgements

257 Not applicable.

258 Use of artificial intelligence tools

259 Large language model tools were used for drafting support, code-execution log sum-
260 marization and language editing. No artificial intelligence tool was listed as an author.
261 The authors verified all analyses, references, figures and manuscript content and take
262 full responsibility for the final work.

263 Additional files

264 Additional file 1. File name: `additional_file_1_harmonization_and_cohort_`
265 `construction.xlsx`. File format: XLSX. Title: Cohort construction, harmoniza-
266 tion and baseline covariate metadata. Description: Workbook containing item-level
267 harmonization crosswalks, cohort tier lock, harmonization risk matrix and baseline
268 clinical/design/covariate availability tables.

269 Additional file 2. File name: `additional_file_2_profile_stability_and_`
270 `descriptive_profiles.xlsx`. File format: XLSX. Title: Descriptive profile and
271 model-stability guardrails. Description: Workbook containing GMM stability sum-
272 maries, 200-refit bootstrap robustness tables, covariance diagnostics, selected class
273 dictionary, descriptive profile summaries, profile-stability guardrail tables, algorithm
274 robustness sensitivity and sensitivity-cohort profile support.

275 Additional file 3. File name: `additional_file_3_lfo_functional_change_`
276 `associations.xlsx`. File format: XLSX. Title: Leave-functional-domain-out
277 functional-change association outputs. Description: Workbook containing decoupled
278 validation comparisons, functional endpoint leakage audit, strict-core LFO association
279 table, sensitivity rows, class-level risks, AUC bootstrap intervals and cross-cohort
280 heterogeneity summaries for functional-change analyses.

281 Additional file 4. File name: `additional_file_4_secondary_endpoints_and_`
282 `prediction_guardrails.xlsx`. File format: XLSX. Title: Secondary endpoint and
283 prediction guardrail outputs. Description: Workbook containing formal mortality
284 guardrail models, hospitalization candidate models/status tables, hard-endpoint fea-
285 sibility matrix, calibration metrics, decision-curve summaries, secondary endpoint
286 prediction guardrails and cross-cohort mortality heterogeneity summaries.

287 Additional file 5. File name: `additional_file_5_survey_design_and_sex_`
288 `comparator_audits.xlsx`. File format: XLSX. Title: Survey-design and sex-
289 comparator audits. Description: Workbook containing survey-design variable
290 and raw/codebook audits, survey triplet status, baseline male-comparator sum-
291 maries/contrasts, all-sex LFO sex-interaction terms and a reviewer-risk summary
292 sheet.

293

294 Supplementary Figures S1-S5 are embedded after the references in this manuscript
295 file and are not separate tabular Additional files.

296 References

297 [1] Lee J, Phillips D, Wilkens J, Chien S, Lin Y, Angrisani M, et al. Gate-
298 way to Global Aging Data: resources for cross-national comparisons of family,
299 social environment and healthy aging. The Journals of Gerontology: Series B.
300 2021;76(Supplement 1):S5–S16. <https://doi.org/10.1093/geronb/gbab050>.

301 [2] World Health Organization. World report on ageing and health. Geneva: World
302 Health Organization; 2015. Available from: [https://www.who.int/publications/](https://www.who.int/publications/i/item/9789241565042)
303 [i/item/9789241565042](https://www.who.int/publications/i/item/9789241565042).

304 [3] World Health Organization. Integrated care for older people: guidelines on
305 community-level interventions to manage declines in intrinsic capacity. Geneva:

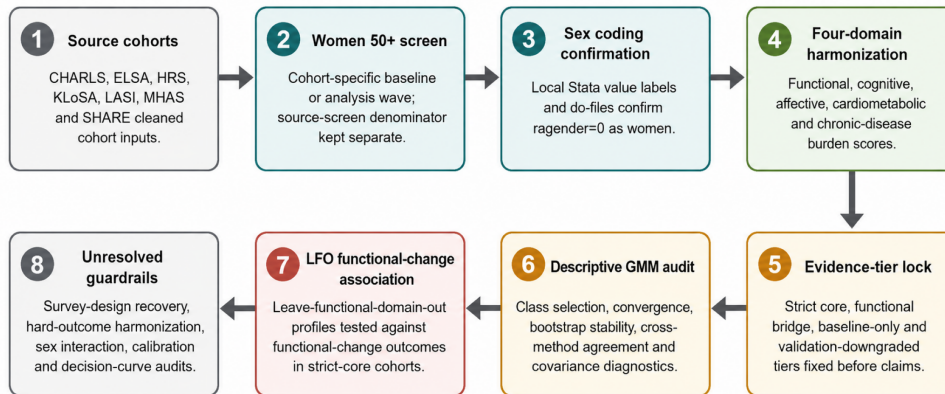
- World Health Organization; 2017. Available from: <https://iris.who.int/handle/10665/258981>.
- [4] Cesari M, Araujo de Carvalho I, Amuthavalli Thiyagarajan J, Cooper C, Martin FC, Reginster JY, et al. Evidence for the domains supporting the construct of intrinsic capacity. *The Journals of Gerontology Series A, Biological Sciences and Medical Sciences*. 2018;73(12):1653–1660. <https://doi.org/10.1093/geronagly011>.
- [5] Barnett K, Mercer SW, Norbury M, Watt G, Wyke S, Guthrie B. Epidemiology of multimorbidity and implications for health care, research, and medical education: a cross-sectional study. *Lancet*. 2012;380(9836):37–43. [https://doi.org/10.1016/S0140-6736\(12\)60240-2](https://doi.org/10.1016/S0140-6736(12)60240-2).
- [6] Fried LP, Tangen CM, Walston J, Newman AB, Hirsch C, Gottdiener J, et al. Frailty in older adults: evidence for a phenotype. *The Journals of Gerontology Series A, Biological Sciences and Medical Sciences*. 2001;56(3):M146–M156. <https://doi.org/10.1093/gerona/56.3.M146>.
- [7] Rockwood K, Mitnitski A. Frailty in relation to the accumulation of deficits. *The Journals of Gerontology Series A, Biological Sciences and Medical Sciences*. 2007;62(7):722–727. <https://doi.org/10.1093/gerona/62.7.722>.
- [8] Clegg A, Young J, Iliffe S, Rikkert MO, Rockwood K. Frailty in elderly people. *Lancet*. 2013;381(9868):752–762. [https://doi.org/10.1016/S0140-6736\(12\)62167-9](https://doi.org/10.1016/S0140-6736(12)62167-9).
- [9] Crimmins EM, Kim JK, Sole-Auro A. Gender differences in health: results from SHARE, ELSA and HRS. *European Journal of Public Health*. 2011;21(1):81–91. <https://doi.org/10.1093/eurpub/ckq022>.

- 330 [10] Quiñones AR, Nagel CL, Botoseneanu A, Newsom JT, Dorr DA, Kaye J,
331 et al. Multidimensional trajectories of multimorbidity, functional status, cog-
332 nitive performance, and depressive symptoms among diverse groups of older
333 adults. *Journal of Multimorbidity and Comorbidity*. 2022;12:26335565221143012.
334 <https://doi.org/10.1177/26335565221143012>.
- 335 [11] Zhao Y, Hu Y, Smith JP, Strauss J, Yang G. Cohort profile: the China
336 Health and Retirement Longitudinal Study (CHARLS). *International Journal of*
337 *Epidemiology*. 2014;43(1):61–68. <https://doi.org/10.1093/ije/dys203>.
- 338 [12] Steptoe A, Breeze E, Banks J, Nazroo J. Cohort profile: the English Longitudinal
339 Study of Ageing. *International Journal of Epidemiology*. 2013;42(6):1640–1648.
340 <https://doi.org/10.1093/ije/dys168>.
- 341 [13] Sonnega A, Faul JD, Ofstedal MB, Langa KM, Phillips JWR, Weir DR. Cohort
342 profile: the Health and Retirement Study (HRS). *International Journal of*
343 *Epidemiology*. 2014;43(2):576–585. <https://doi.org/10.1093/ije/dyu067>.
- 344 [14] Korea Employment Information Service. Korean Longitudinal Study of Ageing
345 (KLoSA): study design and user guide. Korea Employment Information Service;
346 2026. Accessed 2 June 2026. Available from: [https://survey.keis.or.kr/eng/klosa/](https://survey.keis.or.kr/eng/klosa/index.jsp)
347 [index.jsp](https://survey.keis.or.kr/eng/klosa/index.jsp).
- 348 [15] Perianayagam A, Bloom D, Lee J, Parasuraman S, Sekher TV, Mohanty SK,
349 et al. Cohort Profile: The Longitudinal Ageing Study in India (LASI). *Interna-*
350 *tional Journal of Epidemiology*. 2022;51(4):e167–e176. [https://doi.org/10.1093/](https://doi.org/10.1093/ije/dyab266)
351 [ije/dyab266](https://doi.org/10.1093/ije/dyab266).
- 352 [16] Wong R, Michaels-Obregon A, Palloni A. Cohort Profile: The Mexican Health
353 and Aging Study (MHAS). *International Journal of Epidemiology*. 2017;46(2):e2.

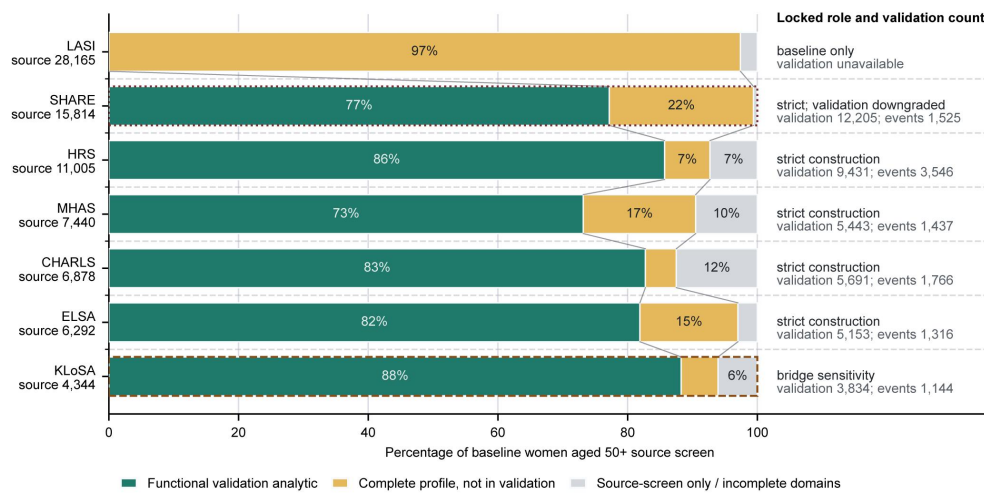
- 354 <https://doi.org/10.1093/ije/dyu263>.
- 355 [17] Borsch-Supan A, Brandt M, Hunkler C, Kneip T, Korbmacher J, Malter F, et al.
 356 Data Resource Profile: the Survey of Health, Ageing and Retirement in Europe
 357 (SHARE). *International Journal of Epidemiology*. 2013;42(4):992–1001. <https://doi.org/10.1093/ije/dyt088>.
 358
- 359 [18] von Elm E, Altman DG, Egger M, Pocock SJ, Gøtzsche PC, Vandenbroucke JP,
 360 et al. The Strengthening the Reporting of Observational Studies in Epidemiology
 361 (STROBE) statement: guidelines for reporting observational studies. *Lancet*.
 362 2007;370(9596):1453–1457. [https://doi.org/10.1016/S0140-6736\(07\)61602-X](https://doi.org/10.1016/S0140-6736(07)61602-X).
- 363 [19] Katz S, Ford AB, Moskowitz RW, Jackson BA, Jaffe MW. Studies of illness
 364 in the aged. The index of ADL: a standardized measure of biological and psy-
 365 chosocial function. *JAMA*. 1963;185:914–919. <https://doi.org/10.1001/jama.1963.03060120024016>.
 366
- 367 [20] Lawton MP, Brody EM. Assessment of older people: self-maintaining and
 368 instrumental activities of daily living. *The Gerontologist*. 1969;9(3):179–186.
- 369 [21] Radloff LS. The CES-D Scale: A self-report depression scale for research in
 370 the general population. *Applied Psychological Measurement*. 1977;1(3):385–401.
 371 <https://doi.org/10.1177/014662167700100306>.
- 372 [22] Prince MJ, Beekman AT, Deeg DJ, Fuhrer R, Kivela SL, Lawlor BA, et al.
 373 Depression symptoms in late life assessed using the EURO-D scale. Effect of age,
 374 gender and marital status in 14 European centres. *British Journal of Psychiatry*.
 375 1999;174:339–345. <https://doi.org/10.1192/bjp.174.4.339>.
- 376 [23] Doiron D, Burton P, Marcon Y, Gaye A, Wolffenbuttel BHR, Perola M, et al. Data
 377 harmonization and federated analysis of population-based studies: the BioSHaRE

- 378 project. *Emerging Themes in Epidemiology*. 2013;10(1):12. [https://doi.org/10.](https://doi.org/10.1186/1742-7622-10-12)
379 [1186/1742-7622-10-12](https://doi.org/10.1186/1742-7622-10-12).
- 380 [24] Fortier I, Raina P, Van den Heuvel ER, Griffith LE, Craig C, Saliba M, et al.
381 Maelstrom Research guidelines for rigorous retrospective data harmonization.
382 *International Journal of Epidemiology*. 2017;46(1):103–105. [https://doi.org/10.](https://doi.org/10.1093/ije/dyw075)
383 [1093/ije/dyw075](https://doi.org/10.1093/ije/dyw075).
- 384 [25] McLachlan GJ, Peel D. *Finite Mixture Models*. New York: John Wiley & Sons;
385 2000.
- 386 [26] Nylund KL, Asparouhov T, Muthen BO. Deciding on the number of classes in
387 latent class analysis and growth mixture modeling: a Monte Carlo simulation
388 study. *Structural Equation Modeling*. 2007;14(4):535–569. [https://doi.org/10.](https://doi.org/10.1080/10705510701575396)
389 [1080/10705510701575396](https://doi.org/10.1080/10705510701575396).
- 390 [27] Hennig C. What are the true clusters? *Pattern Recognition Letters*. 2015;64:53–
391 62. <https://doi.org/10.1016/j.patrec.2015.04.009>.

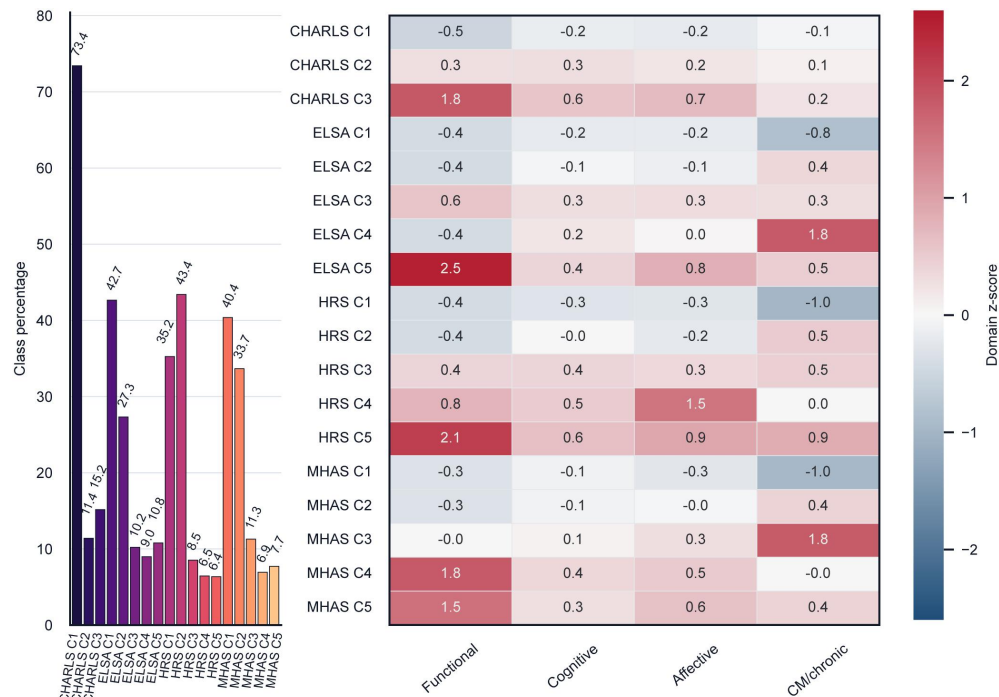
392 **Supplementary figures**



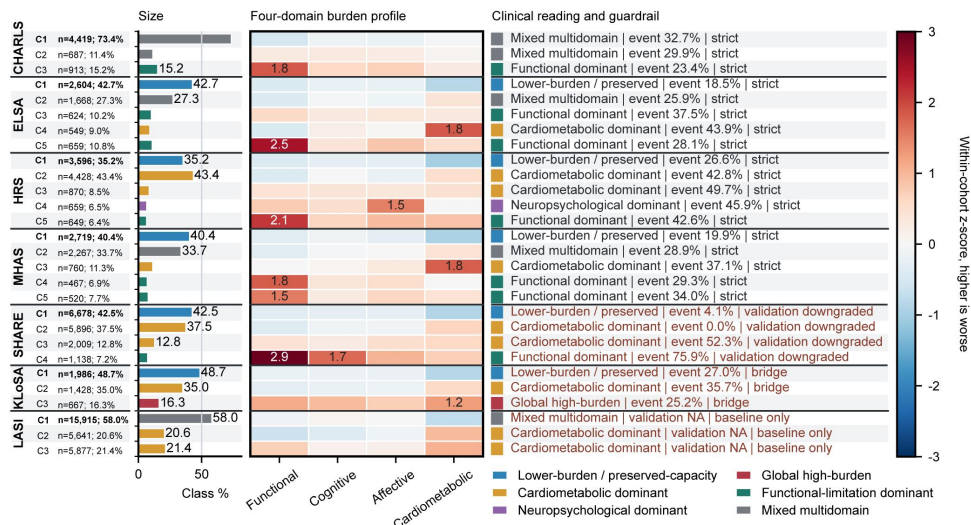
Supplementary Figure S1. Workflow schematic showing the analysis chain from source cohorts through women aged 50 years or older screening, sex-coding confirmation, four-domain harmonization, evidence-tier locking, descriptive GMM audit, leave-functional-domain-out functional-change association and unresolved guardrail audits.



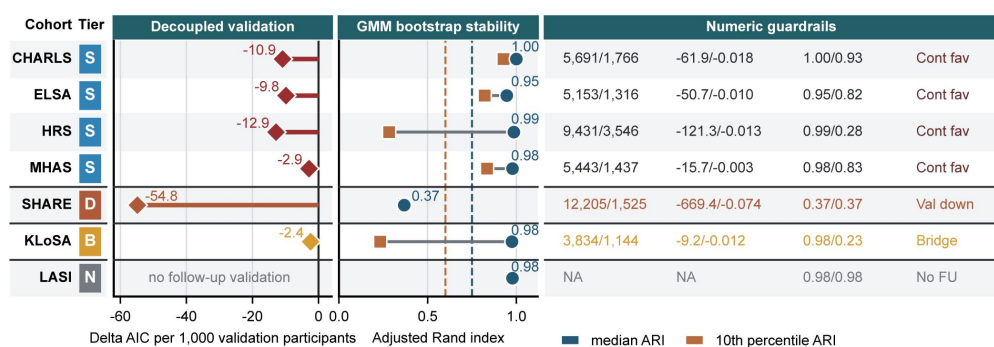
Supplementary Figure S2. Cohort denominator and validation dashboard showing source women aged 50 years or older, complete-domain analytic proportions, LFO validation availability and evidence-tier labels.



Supplementary Figure S3. Strict-core descriptive profile heatmap and class-size display for CHARLS, ELSA, HRS and MHAS.



Supplementary Figure S4. Full descriptive profile heatmap and clinical-reading panel across strict-core, sensitivity and baseline-only tiers.



Supplementary Figure S5. Decoupled validation, GMM bootstrap stability and numerical guardrail dashboard retained as supplementary context.